

Calendar Line 3

Case Name: *Esther Mayora vs Volkswagen Group of America, Inc.*

Case No.: 24CV450793

Plaintiff has filed a late opposition to Defendant's motion for an order compelling Plaintiff to appear for deposition by remote electronic means and to produce documents or in the alternative continue trial. To some extent, the Court understands the confusion. Subsequent to the filing of the motion, the Court, by separate application, did continue the trial date. The order continuing trial, however, was based on Defendant's request that the Court *either advance* the hearing on this motion *or* continue the trial. The Court continued the trial. The Court did not advance the hearing on this motion, thereby leaving this motion on calendar for June 10, 2026.

Although, Defendant did not withdraw the instant motion as moot, under the circumstances here, the Court finds that it is moot. The Court is aware that as a part of a separate stipulation and proposed order, the parties indicated that "Plaintiff's counsel is in the process of confirming earliest availability to produce Plaintiff for deposition." Trial is now set for February 16, 2027. So long as Plaintiff, in good faith, properly sits for deposition within the parameters of the currently set trial date, Court involvement should not be necessary.

The Court will prepare the final order.

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Calendar Line 4 & 5

Case Name: *Vincent Marino, Jr. v. County of Santa Clara*

Case No.: 24CV453955

Before the Court is Defendant County of Santa Clara's ("Defendant" or the "County") demurrer to the First Amended Complaint ("FAC") filed by Plaintiff Vincent Marino, Jr. ("Plaintiff") and motion to strike portions contained therein.

This is an action for negligence. According to the allegations of the FAC, on January 1, 2024, Plaintiff was admitted to the Santa Clara Valley Medical Center (the "Medical Center") where he presented with persistent nausea, lightheadedness, shortness of breath, and black stool. (FAC, ¶¶ 16-17.) Plaintiff had a history of chronic kidney disease and diabetes and was found to be suffering from severe sepsis. (*Id.*, ¶ 17.) While he was admitted, Plaintiff suffered from a cerebrovascular accident due to embolism of the left vertebral artery, which resulted in oropharyngeal dysphagia, apraxia, speech and language deficits and aphasia. (*Ibid.*) He also suffered from metabolic encephalopathy and stroke, was intubated, and required a gastrostomy tube for nutrition and hydration. (*Id.*, ¶ 25.)

Defendants knew that Plaintiff was dependent upon the Medical Center for medical and custodial care. (FAC, ¶ 18.) The Medical Center's failure to provide adequate assistance caused Plaintiff to suffer significant injuries, including a Stage IV pressure ulcer, which Plaintiff was at a high risk for developing. (FAC, ¶¶ 20, 25.) Because of his condition, Plaintiff required physical assistance with bed mobility to relieve pressure on areas of the body susceptible to developing pressure wounds. (*Id.*, ¶ 25.)

Defendants failed to continually assess and monitor Plaintiff for skin breakdown or changes in health condition, failed to keep him clean and dry, failed to reposition him every two hours, and failed to provide discharge planning and instruction relative to the pressure wound. (FAC, ¶ 26.) As a result, Plaintiff developed a serious pressure ulcer to his sacrum which progressed to a Stage IV pressure ulcer. (*Id.*, ¶¶ 25, 28.)

On January 8, 2024, wound care nurse, Matthew Lee, R.N. ("Lee") was consulted for moisture associated skin damage to Plaintiff's buttocks. (FAC, ¶ 29.) Lee did not visualize the wound, as Plaintiff was being transported to the radiology department for an MRI. (*Ibid.*) A plan was implemented to care for Plaintiff's skin. (*Ibid.*)

On January 9, 2024, another wound care nurse evaluated Plaintiff and noted the wound was dark brown, there was skin loss and drainage, and Plaintiff was suffering from frequent loose stool and urine. (FAC, ¶ 30.) Despite recommendations, the Medical Center failed to reposition Plaintiff and he was left four hours soiled, which resulted in further skin breakdown. (*Id.*, ¶ 31.) Plaintiff was not seen by wound care again until February 1, 2024 and that time, the top layers of skin on Plaintiff's coccyx and buttocks had necrosed. (*Id.*, ¶ 32.)

On February 7, 2024, Dr. Kelly Higa ("Dr. Higa") evaluated Plaintiff for possible debridement of the large sacral ulcer and Dr. Higa noted that the ulcer was due to immobility and recommended wound care and collagenase treatment. (FAC, ¶ 33.) The wound was debrided on February 14, 2024. (*Ibid.*)